Emergent heterogeneity in compartmental disease models and its relation to super-spreading

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Abstract

Heterogeneity in infectious disease spread can reshape the risk and severity of outbreaks. This heterogeneity can arise from intrinsic individual-level differences in susceptibility, contact patterns and infectious durations. Although intrinsic heterogeneity has been examined in depth, heterogeneity can also arise as a consequence of the disease dynamics. Changes in the size of susceptible populations can lead to emergent heterogeneity, i.e., differences in realized variation in the number of secondary cases caused by a primary case. Here, we investigate the magnitude and mechanisms generating emergent heterogeneity by analyzing secondary case distributions within the canonical SIR model. We first describe κ, a statistic for quantifying the emergent heterogeneity in secondary case counts in the SIR model and stochastic realizations. Although intrinsic heterogeneity decreases systematically with $\mathcal{R}_0$, we show that κ is invariant for all values of $\mathcal{R}_0$ within stochastic SIR models and we prove this result mathematically. The invariance of emergent heterogeneity in SIR models results from distinct partitioning of within- and between-cohort components of κ. Our results demonstrate that heterogeneity emerges even in simple SIR epidemics as a result of feedback between stochastic depletion of susceptibles and subsequent opportunities of transmission. More broadly, our results suggest that efforts to understand the basis for realized variation in transmission should account for the impacts of susceptible depletion beyond the narrow limits of the initial outbreak phase.

1 Introduction

Real outbreaks have significant heterogeneity in the number of secondary cases – including superspreading, marked by a small fraction of individuals causing a disproportionately large fraction of realized transmission [13, 19]. Superspreading can help explain why epidemics take off in some situations and not in others, despite apparently similar contexts [12, 4]. Leveraging knowledge of superspreading can make control efforts more effective [5, 15, 18]. It is especially

important for disease dynamics and control strategies at the beginning of outbreaks [6]. Correspondingly, theoretical treatment of superspreading emphasizes individual-level transmission patterns, often while assuming a fully susceptible population, [13, 9] and has focused on models that link observed secondary case counts to a continuous random variable, the *individual reproductive number*, via a Poisson sampling process. Canonically, the individual reproductive number is modeled as a gamma distribution, giving rise to negative binomial secondary case distributions [13]. In this way, variation arising from the stochastic transmission process is conceptually separated from *intrinsic heterogeneity* in expected infectiousness that may emerge from potential differences in biology or behaviors between individuals [3, 1, 7].

However, these models assume a constant pool of susceptible individuals and do not account for disease dynamics beyond the very initial stages of an outbreak. Over longer timescales, the susceptible depletion and replenishment drive population-scale disease dynamics. Changes in the size of susceptible population changes the statistics of the contact processes that lead to successful disease transmission. As a result, variation in realized secondary cases arises between early and late infectors, even when the underlying individual reproductive number distribution is identical. In practice, many deterministic epidemiological models also include *explicit heterogeneity*, for example by adding compartments for different demographic or geographic groups [17], which can have different average rates of mixing, transmission, or recovery. When implemented in terms of individual events, the inclusion of explicit heterogeneity reproduces superspreading in linear compartmental models [14]. Despite shared theory motivating models that highlight individual-level transmission, and those that emphasize population-level transmission dynamics, a gap persists in how each model framework treats individual-level transmission heterogeneity.

The canonical SIR (Susceptible-Infected-Removed) model uses ordinary differential equations (ODEs) to describe disease dynamics. It assumes constant, uniform infectivity and recovery times across all individuals during the infectious stage [8, 10]. Although it appears homogeneous, even the canonical SIR model has two sources of *emergent heterogeneity.* The changing effective reproductive number arising from susceptible dynamics, drives variation between fractions of the population infected at different times; and the exponentially distributed recovery times assumed by the ODE drive variation in the number of infections attributable to fractions of the population infected at a given time. In other words, even the fully deterministic SIR model predicts variation in the expected number of secondary cases.

Here we delineate sources of individual variation in transmission inscribed in simple dynamical models. We trace how these sources compose into observed or observable heterogeneity in different settings, for example contrasting how heterogeneity emerges in dynamical models versus linearized, or branching-process models. We begin by revisiting early findings that intrinsic heterogeneity (e.g., due to differences in the basic reproductive number, $\mathcal{R}_0$) drives differences in variation in the secondary case distribution. However, when accounting for the feedback of transmission on disease dynamics, we find that emergent hetero-

geneity is an invariant across a full SIR outbreak and independent of differences in intrinsic heterogeneity. As we show, expected differences in superspreading may be a consequence, in part, of focusing on early rather than the full scope of outbreaks. Throughout, we highlight the understudied but critically important role of disease dynamics *per se* in generating individual-level heterogeneity. We aim to spur the development of theory and methods that better integrate heterogeneity and dynamics, ultimately facilitating feedback between data on incidence and individual-level transmission chains, mechanistic model specification, identification, validation, and intervention design.

2 Results

2.1 Heterogeneity in the secondary case distribution in the linearized SIR model

The early outbreak phase of an S(usceptible)-I(nfectious)-R(emoved) epidemic is modeled using a linearized SIR model, which assumes that the pool of susceptible individuals is very large and does not change appreciably at the outset of an epidemic. The early dynamics of the epidemic can be characterized by a transmission rate and an infectious period. Demographic stochasticity generates heterogeneity in this model despite the lack of explicit accounting of individual-level variation in transmission rates and infectious periods. The model implicitly assumes that the infectious period follows an exponential distribution which results in an exponentially distributed expected infectiousness (or *individual reproductive number*) with mean $\mu = \mathcal{R}_0$ and dispersion $\kappa = 1$. Infectious individuals spread the disease by coming into contact with susceptible (and other infectious individuals), which is modelled as a Poisson process. The resulting distribution of secondary cases (i.e., the number of new cases caused by a single infectious individual over their infectious lifetime) is geometric and has a mean equal to $\mathcal{R}_0$. Thus, the expected infectious and secondary case distributions are characterized by $\mathcal{R}_0$ (Figure Figure 1a).

The heterogeneity in the secondary case distribution means that a large fraction of the initial secondary cases are generated by only a few infectors. Due to the Poisson contact process, the inequality curve for the realized secondary case distribution deviates from the inequality curve of the exponentially distributed expected infectiousness, in an $\mathcal{R}_0$ dependent manner (Figure 1b). As $\mathcal{R}_0$ increases, the inequality observed in the secondary case distribution decreases and approaches the expected infectiousness inequality curve (Figure 1b). This heterogeneity arises from the implicit underlying differences in expected infectiousness and the stochasticity of the transmission process. As $\mathcal{R}_0$ becomes smaller, an increasing fraction of infected individuals generate no secondary infections, so the individuals who do trans- mit make up a smaller fraction of all infectors. Indeed, the 20/80 rule that 80% of secondary infections arise from only 20% of infectious individuals is attained when $\mathcal{R}_0$ falls just below 0.4. [AA: don't know why] However, even when $\mathcal{R}_0$ exceeds the epidemic threshold, the

distribution of secondary cases can still display substantial heterogeneity. However, as the epidemic progresses, we must account for emergent heterogeneity as the pool of susceptible individuals shrinks.

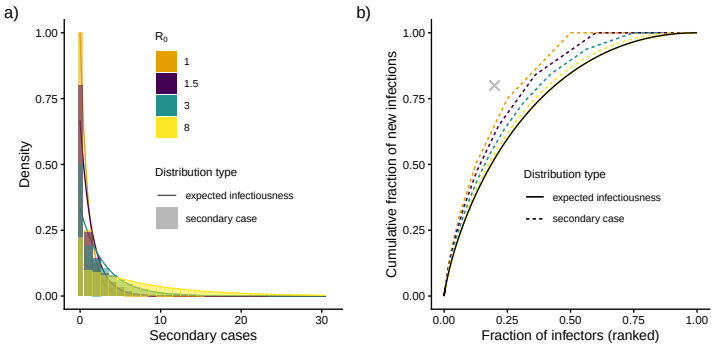


Figure 1: **Heterogeneity in the linearized SIR model** due to implicit variation in recovery times among infectors. (a) Both expected infectiousness (density curves) and secondary case distributions (density histograms) for the outset of an SIR epidemic, where the population is fully susceptible. (b) Inequality curves, defined as the cumulative fraction of new infections versus the cumulative fraction of infectors. Curves (in black) for the expected infectiousness across varying $\mathcal{R}_0$ coincide. Inequality curves derived from the secondary case distributions shown with dashed lines. The cross marks "20-80" point, where 20% of infectors cause 80% of infections. In panel a, because the first bin (at zero) sits at the boundary of support for each distribution, we have plotted this bin as double the density and half the width; this adjustment preserves area-to-area correspondence with the probability density function, while facilitating visual comparison of the heights of the density and mass functions.

2.2 Heterogeneity in secondary cases over an epidemic is independent of $\mathcal{R}_0$ - simulation

Next we consider the impact of depletion of susceptible population during outbreaks on secondary case distributions. We simulated stochastic SIR epidemics with different $\mathcal{R}_0$ values in a population of 10000 individuals. For each infected individual, we recorded the number of secondary cases they generated during the outbreak. Figure 2 shows realized distributions of secondary cases caused by individual infectors over the entire outbreaks. Unlike in the linearized scenario shown in Figure 1, where the susceptible pool remains fully populated, the secondary case distributions, computed across the entire outbreak, appear to be indistinguishable across a broad range of the key parameter $\mathcal{R}_0$ (compare panels (a) of Figure 1 and Figure 2). This result is unexpected. In the absence of susceptible depletion, heterogeneity is expected to be larger for smaller values

of $\mathcal{R}_0$ as shown in Figure 1b. However, faster and larger dynamical changes in susceptible depletion during the epidemic occur when $\mathcal{R}_0$ is large (Figure S1), which drives larger variation in the expected infectiousness distribution and, hence, secondary-case distribution. The observed stochastically realized secondary case distribution shown in Figure 2 includes multiple notable features. First, the mean of the secondary case distribution is approximately 1. Because we are considering a single outbreak seeded by single initial case, then every case must have its origin within the outbreak. Second, **[TG:** *the ensamble average of*] the variance of the secondary case distribution is equal to 1, consistent with the emergence of what appears to be an exponential distribution in the secondary case distribution considered across an epidemic, independent of $\mathcal{R}_0$.

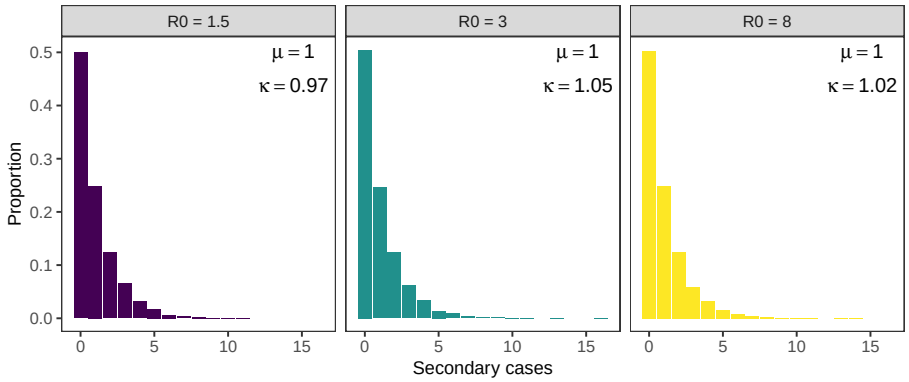


Figure 2: **Heterogeneity in the number of secondary cases over the entire epidemic is independent of $\mathcal{R}_0$.** Histograms of secondary cases realized from individual-based simulations of the SIR model across different $\mathcal{R}_0$s. Simulations performed for a total population of 10^4 individuals with one initial infectious individual. μ and κ denote the mean and dispersion of the secondary case distribution obtained from a single realization of the individual-based model for each $\mathcal{R}_0$.

Box 1. Linking variance across discrete and continuous time models

During the early stages of an outbreak, secondary-cases counts are typically modeled using a negative-binomial distribution, with mean μ and shape, or "dispersion," parameter k. This distribution is linked through Poissonification to a Gamma distribution with the same mean and shape parameter values. Here, the Gamma distribution describes variation among individuals in expected infectiousness.

We note a couple of points. First, the squared coefficient of variation (SCV) of the underlying Gamma distribution is $\kappa = 1/k$, so its variance is $\sigma^2_{\text{act}} = \kappa\mu^2$. The resulting variance in secondary-case counts is $\sigma^2 = \kappa\mu^2 + \mu$, with the last term arising from the Poisson step of the negative-binomial construction. Second, the relationship between σ^2_{act} and σ^2 holds regardless of the form of the expected infectiousness distribution [11].

As κ approaches zero, the distribution of expected infectiousness converges to a point mass at μ, while the corresponding distribution of secondary-case counts converges to a Poission distribution with mean μ. We therefore use κ, as a (generalized) *dispersion* parameter, with larger values indicating greater heterogeneity in expected infectiousness and secondary cases.

2.3 Heterogeneity in secondary cases over an epidemic is independent of $\mathcal{R}_0$ - Proof

We set out to explore the basis for the finding that both the mean and variance of secondary cases in the stochastically realized SIR model is invariant with respect to changes in $\mathcal{R}_0$. To do so, consider the deterministic ODE-based SIR model:

$$\begin{aligned} \frac{dx}{dt} &= -\mathcal{R}_0 x(t) y(t), \\ \frac{dy}{dt} &= \mathcal{R}_0 x(t) y(t) - y(t), \end{aligned} \tag{1}$$

where $x(t)$ is the proportion of susceptible individuals at time t, $y(t)$ is the proportion of infectious individuals at time t, t is time measured in units of the mean infectious duration and $\mathcal{R}_0$ is the basic reproduction number.

To calculate the mean and variance of the secondary cases, we must track infections and the infectors that caused those infections. While individual infectors and cases cannot be tracked in this deterministic SIR model, we can track cohorts of infectors and the cases caused by them. By calculating the weighted moments of the secondary cases per case caused by each cohort, we can obtain the heterogeneity of the secondary cases per case over the epidemic.

We define a cohort, c_{τ_1,τ_2}, as the fraction of infectors that become infectious at time τ_1 and stop being infectious at time τ_2. The weight of the cohort,

$w(\tau_1, \tau_2)$ is the number of infectors in the cohort. The number of infectors who become infectious at time τ_1 is $i(\tau_1) = \mathcal{R}_\prime x(\tau_1)y(\tau_1)$. This quantity, $i(\tau_1)$ is also called the incidence of infection, or the number of new cases that appear at time τ_1. Only a fraction of these infectors remain infectious till time τ_2. If $f(.)$ denotes the probability density function of the infectious period, then

$$w(\tau_1, \tau_2) = i(\tau_1)f(\tau_2 - \tau_1) \tag{2}$$

Now that we have the cohort weights, we need to calculate the number of cases per case caused by the cohort. The total number of cases caused by the cohort c_{τ_1,τ_2} over the course of the epidemic is

$$\hat{C}(\tau_1, \tau_2) = \int_{\tau_1}^{\tau_2} \mathcal{R}_0 x(t')w(\tau_1, \tau_2)dt' = \mathcal{R}_0 w(\tau_1, \tau_2) \int_{\tau_1}^{\tau_2} x(t')dt' \tag{3}$$

Then the secondary cases per case caused by cohort c_{τ_1,τ_2} is

$$C(\tau_1, \tau_2) = \frac{\hat{C}(\tau_1, \tau_2)}{w(\tau_1, \tau_2)} = \mathcal{R}_0 \int_{\tau_1}^{\tau_2} x(t')dt' \tag{4}$$

Now that we have the number of secondary cases per case and weight for each cohort, we can calculate the moments of the secondary cases per case distribution. The kth weighted raw moment of the secondary cases per case distribution, C_k is

$$C_k = \iint\limits_{0<\tau_1\leq\tau_2<\infty} d\tau_1 d\tau_2 \; w(\tau_1, \tau_2)(C(\tau_1, \tau_2))^k, \tag{5}$$

With this definition of the raw moments, the assertion that the heterogeneity of the secondary case per case distribution over the entire epidemic is invariant is equivalent to:

$$\kappa = \frac{C_0 C_2}{C_1^2} - 1 = 1 \tag{6}$$

The detailed calculation of the moments and the proof can be found in ([**TG:** *(refer to the appropriate section)*]). We note that the result follows from two assumptions: (a) the distribution of the infectious duration is memoryless, and (b) the initial infectious cohort is infinitesimal, i.e., $y(t = 0) \to 0$. The first assumption is true for the canonical SIR model where the duration of infectious periods is exponentially distributed. The second assumption is generally true when considering large naive populations in which a very small number of infectious individuals are introduced. Intuitively, C_0 is the total number of cases over the course of the epidemic and C_1 is the total number of *secondary* cases over the course of the epidemic. When the initial population of infectious individuals is infinitesimally small, all the cases can be attributed as secondary cases of some infectious individual who became infectious during the course of the epidemic and thus $C_0 = C_1 = Z$. ([**TG:** *(Is there an intuitive explanation for why C2 is*

what it is?)]). However, if the initial proportion of infectious individuals is not infinitesimally small, this equality no longer holds. The difference between C_0 and C_1 quantifies the size of the initial infectious cohort and C_2 deviates from $2Z$ with a deviation proportional to the size of the initial infectious population (refer to the SI section).

2.4 Balance between within- and between-cohort components of the variance in secondary cases explains why heterogeneity in secondary cases is independent of $\mathcal{R}_0$

To understand why the heterogeneity in secondary cases over the course of the epidemic is independent of $\mathcal{R}_0$, we split the heterogeneity into two components: a within-cohort component and a between-cohort component. A cohort refers to a group of individuals that become infectious at the same time (TG: make vertical slices in Figure 3a to mark cohorts). Individuals within a cohort have access to the same susceptible pool initially but have differences in their secondary case counts as a result of their exponentially distribution infectious periods. Thus, the within-cohort variance can be attributed to inherent individual-level differences in infectious periods. On the other hand, cohorts that arise at different times have realized differences in the number of secondary cases due to differences in the size of the susceptible pools that they have access to. It is important to note that both early and late cohorts have the same distributions of infectious periods and expected infectiousness but due to the depletion of the susceptible pool over time (Figure 3b), later cohorts, on average, have much lower realized secondary cases than earlier cohorts. Thus, the between-cohort variance can be attributed to susceptible depletion.

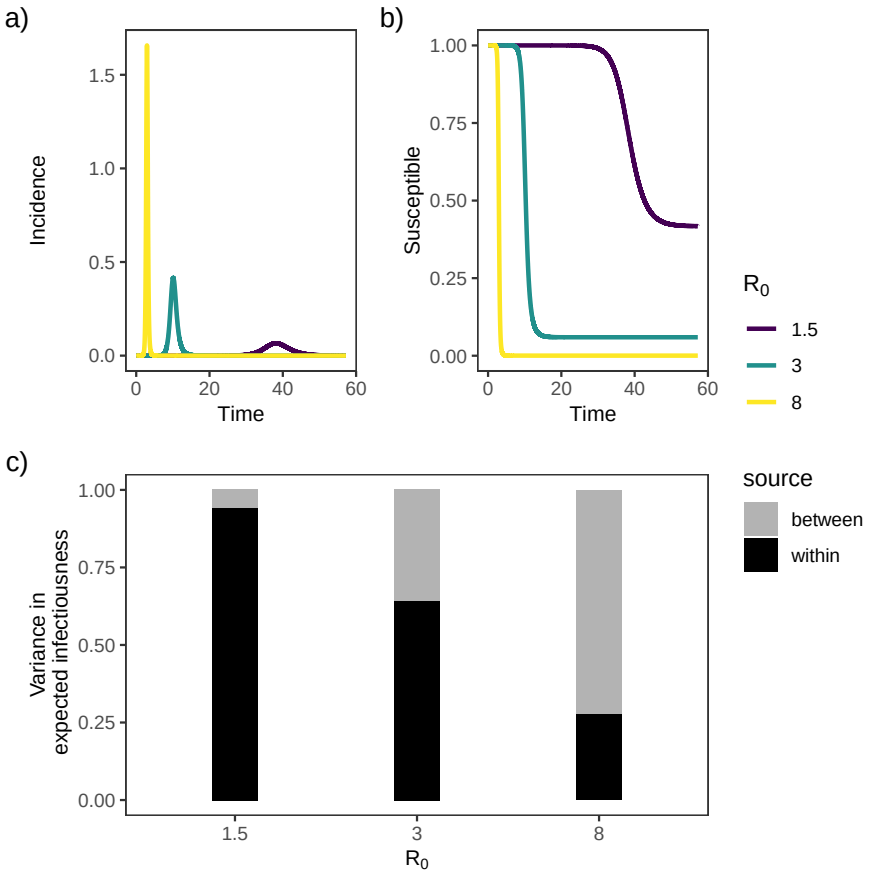


Figure 3: **Deterministic SIR dynamics and within- and between-cohort variance** (a) Incidence and (b) proportion of susceptibles as functions of time, for different values of $\mathcal{R}_0$. The epidemics were modeled using an ODE-based SIR model. Time is in units of mean infectious period.(c) Stacked bar plot showing within- and between-cohort components of variance in secondary cases calculated numerically for different values of $\mathcal{R}_0$.

Numerical calculation of the within- and between-cohort components of the variance in secondary cases using the ODE-based SIR model (see methods for details of the calculation) shows that as $\mathcal{R}_0$ increases, the between-cohort component of the variance increases and the within-cohort component of the variances decreases (fig 3c). However, the sum of the two components, remains constant across $\mathcal{R}_0$. As $\mathcal{R}_0$ increases, the variance in the size of infectious cohorts (fig 4a) and well as the speed and severity of susceptible depletion also increases (fig 3b). As a result of the susceptible depletion, the mean number of secondary cases also decreases rapidly across cohorts (fig 4b), with earlier co-

horts having a much higher mean of secondary cases compared to later cohorts. Further, the decline in the mean number of secondary cases across cohorts becomes sharper as $\mathcal{R}_0$ increases. As a result, the between-cohort variance in the number of secondary cases increases with $\mathcal{R}_0$.

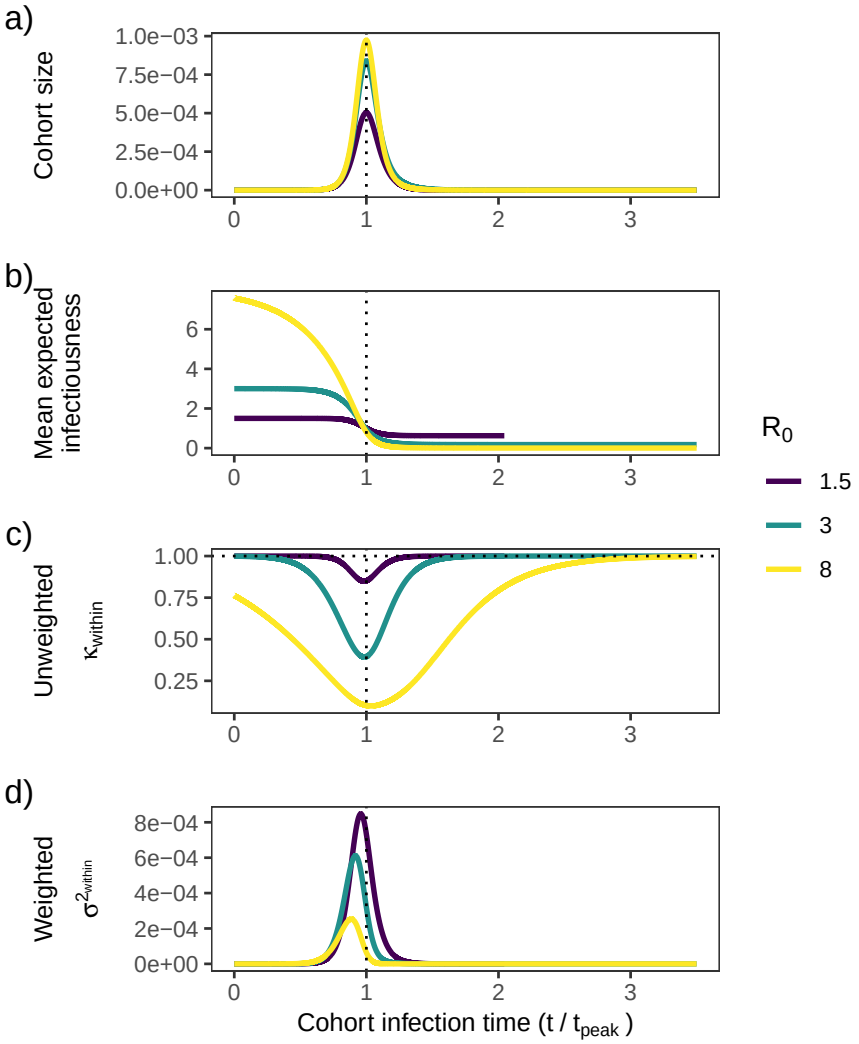


Figure 4: **Expected infectiousness of cohorts.** (a) Cohort size as a function of time of infection. Time has been normalized by the time of peak incidence (t_{peak}). Individuals infected during $[t, t + \Delta t]$ form a cohort, with $\Delta t = 2 \times 10^{-4} t_{\text{peak}}$. (b) Mean expected infectiousness for each cohort. (c) Within-cohort dispersion of secondary cases. (d) Weighted within-cohort dispersion of secondary cases. Note that the x-axes in these plots are not real time. Instead they refer to cohort infection time. For example, the mean expected infectiousness at $t/t_{\text{peak}} = 1$ does not refer to the mean expected infectiousness at time $t = t_{\text{peak}}$. Instead, it refers to the mean expected infectiousness of the cohort that became infectious at time $t = t_{\text{peak}}$.

The within-cohort variance, on the other hand, decreases as $\mathcal{R}_0$ increases (fig 3c). Since the mean number of secondary cases varies widely between early and late cohorts, we look at the within-cohort dispersion in secondary cases to better understand how within-cohort heterogeneity changes between early vs late cohorts for an epidemic with a given $\mathcal{R}_0$. For a given $\mathcal{R}_0$, early cohorts have high dispersion in secondary cases, driven by access to a large susceptible pool (fig 4c). As the epidemic progresses, the susceptible pool depletes (fig 3b) and the within-cohort dispersion in secondary cases decreases (fig 4c). The within-cohort dispersion increases for the later cohorts, this time driven by the shrinking within-cohort mean in the number of secondary cases (fig 4c). However, the within-cohort contribution to the total variance depends not just on the within-cohort dispersion, but also on the cohort weight. Once the contributions of each cohort to the variance are appropriately weighted by the cohort mean and cohort size, we find that the weighted within-cohort dispersion actually increases and decreases with the cohort size (fig 4d) for a given $\mathcal{R}_0$. Indeed, across different values of $\mathcal{R}_0$, we find that the weighted within-cohort dispersion decreases as $\mathcal{R}_0$ increases.

Together, the decrease in the within-cohort variances balances the increase in the between-cohort variance, leading to the total variances in secondary cases being independent of $\mathcal{R}_0$. This change in the within- and between-cohort variances is driven by susceptible depletion.

3 Discussion

Quantifying heterogeneity in secondary-case distributions helps better understand how epidemics unfold and how they can be contained. Although there have been efforts to both quantitatively describe individual-level heterogeneity in contact rates [19] and to parameterize biologically realistic secondary case distributions, abstracted from the mechanism [13], these efforts have been largely separate from the construction and evaluation of the compartmental models that enable epidemic forecasting and intervention design (but see [14]). Here, we expand the bridge between individual- and population perspectives of the heterogeneity by studying epidemiologically-relevant cohorts, those infected at the same time.

We observed that early in the outbreak, mean and variance of the expected infectiousness vary by $\mathcal{R}_0$, so do those of secondary-case distributions. However, when computed across the entire outbreak, these quantities become independent of $\mathcal{R}_0$. We indeed prove that, provided a negligible fraction of initial infectors, both mean and variance are equal to one, regardless of $\mathcal{R}_0$. Disentangling the within- and between-cohort variances reveals that in epidemics with large $\mathcal{R}_0$, unlike those with smaller $\mathcal{R}_0$, the between-cohort variance dominates the within-cohort variance.

In real-time analyses, κ, used as a measure of heterogeneity, remains below one.

There are some limitations to our work. The underlying model is based on a closed, well-mixed population. It is not clear how heterogeneity in mixing propensity might impact the independence of heterogeneity in expected infectiousness from $\mathcal{R}_0$ and, hence, alter κ. Working with SIR models implicitly implies an exponential infectious period. Investigating the impact of infectious period distribution on the heterogeneity in expected infectiousness is left for future work.

[TG: *Can we make a note about for epidemics with large $\mathcal{R}_0$, if you don't start tracking cases right from the beginning, you'll already underestimate cases/case* **] [JD:** *Yes, this should go into the paper.*]

In SIR-type epidemics, the effective reproduction number declines as the susceptible population becomes depleted. Consequently, if surveillance begins after the initial transmission generations, estimates of secondary-cases will represent the reduced effective reproduction number rather than $\mathcal{R}_0$ [16]. As a result, delayed surveillance may yield substantially lower estimates, especially when $\mathcal{R}_0$ is large, due to rapid depletion of the susceptible pool.

4 Materials and Methods

4.1 Individual based SIR model

[TG: *Need details of the Individual based SIR simulations here*]

4.2 Deterministic SIR model

The deterministic ODE-based SIR model is:

$$\begin{aligned} \frac{dS}{dt} &= -\beta \frac{SI}{N}, \\ \frac{dI}{dt} &= \beta \frac{SI}{N} - \gamma I, \\ \frac{dR}{dt} &= \gamma I \end{aligned} \tag{7}$$

, where $S(t)$, $I(t)$ and $R(t)$ are the number of susceptible, infectious and recovered/removed individuals at time t respectively. $N = S + I + R$ is the total number of individuals. β is the transmission rate and γ is the recovery rate. We do not need to track R explicity, since $R = N - S - I$ and N is a constant ($\frac{dN}{dt} = 0$). We rewrite the equations in terms of the rescaled variables, $x = \frac{S}{N}$, $y = \frac{I}{N}$ and $\tau = \gamma t$, to get:

$$\begin{aligned} \frac{dx}{d\tau} &= -\mathcal{R}_0 xy, \\ \frac{dy}{d\tau} &= \mathcal{R}_0 xy - y, \end{aligned} \tag{8}$$

where $\mathcal{R}_0 = \frac{\beta}{\gamma}$ is the basic reproduction number. $x(\tau)$ is the proportion of susceptible individuals, $y(\tau)$ is the proportion of infectious individuals and τ is time in units of the mean recovery period $\frac{1}{\gamma}$.

4.3 Calculation of cohort mean and within-cohort variance of secondary cases

The values of secondary cases in panel (a) of Figure 1 were generated by computing the geometric probability mass function with mean $\mathcal{R}_0$ at points $0, 1, \ldots, 30$. We used the convention that the geometric distribution counts the number of failures before the first success. As for the expected infectiousness, we evaluated the exponential probability density function with mean $\mathcal{R}_0$ at 300 equally distanced point in the interval $[0, 30]$.

To generate panel (b) of Figure 1, we followed the Lloyd-Smith's approach and computed the inequality in the activity distribution [13, SI, Sec. 2.2.5]. More specifically, we used the relation $F_{\text{trans}}(x) = \frac{1}{\mathcal{R}_0^2} \int_0^x v e^{-\frac{v}{\mathcal{R}_0}} dv$ which is the fraction of cases due to those caused up to x secondary cases. The fraction of cases due to those caused more than x cases would be $1 - F_{\text{trans}}(x)$, which is equal to $(1 + \frac{x}{\mathcal{R}_0}) e^{-\frac{x}{\mathcal{R}_0}}$. The population fraction of the individuals infected more than x cases is $e^{-\frac{x}{\mathcal{R}_0}}$. We used a similar approach to compute the inequality in the secondary case distribution: We used the relation $F_{\text{trans}}(x) = \frac{1}{\mathcal{R}_0} \sum_{v=0}^{x} v G(v, \mathcal{R}_0)$ which is the fraction of cases due to those caused up to x secondary cases. Here $G(v, \mathcal{R}_0)$ is a geometric distribution with mean $\mathcal{R}_0$. The fraction of cases due to those caused more than x cases equals $1 - F_{\text{trans}}(x)$. The population fraction of

these individuals would be $1 - P(x)$, where $P(x)$ is the cumulative distribution function of a geometric distribution with mean $\mathcal{R}_0$ evaluated at x.

To generate panel (a) of Figure 2, we used the individual based simulation and computed the number of secondary cases in a population of size 10^4. The simulation was initiated with one case.

We used a simple deterministic SIR model to compute the mean and variance of expected infectiousness $\mathcal{R}_c$. First, we scaled time by the mean infectious period, so the resulting SIR model then depends on one parameter, $\mathcal{R}_0$. The SIR differential equations were numerically solved for the proportions of susceptible $x(t)$ and infectious $y(t)$ at each time point t. We then used the `R` function `approxfun` to construct a function, $X(t)$, that linearly interpolates the time-series $(t, \mathcal{R}_0 x(t))$. We associated a cohort to each of the first 60% time points in the time-series t (We used a 60% cutoff to ensure cohorts had almost recovered by the end of the simulation period) and calculated the cohort-specific mean and variance.

More specifically, for the cohort infected at time point τ, the mean, $\mu(\tau)$, and variance $\sigma^2(\tau)$ of $\mathcal{R}_c$ read as:

$$\mu(\tau) = \int_{t>\tau} f(t-\tau) \int_{\tau}^{t} \mathcal{R}_0 x(s) ds \, dt \,, \tag{9a}$$

$$\mathbb{E}[\mathcal{R}_c^2(\tau)] = \int_{t>\tau} f(t-\tau) \left(\int_{\tau}^{t} \mathcal{R}_0 x(s) ds \right)^2 dt \,, \tag{9b}$$

$$\sigma^2(\tau) = \mathbb{E}[\mathcal{R}_c^2(\tau)] - \mu^2(\tau). \tag{9c}$$

Here $f(\cdot)$ is the probability density function of the infectious period. To calculate these quantities for the cohort infected at τ, the interpolating function $X(t)$ was integrated from τ to t, which is the case reproductive potential associated with the cohort fraction recovered at time point t. The mean $\mathcal{R}_c$ for each cohort $\mu(\tau)$ was then the integral of the case reproductive potential weighted by the infectious period density function. We used the same approach to compute $\mathbb{E}[\mathcal{R}_c^2(\tau)]$ and variance of $\mathcal{R}_c$ for each cohort $(\sigma^2(\tau))$. In Figure 4, the middle panel (panel b) was generated using this approach.

We obtained mean expected infectiousness $\mathcal{R}_c$ by integrating the cohort-specific mean $\mu(\tau)$ against the incidence, $i(\tau) = \mathcal{R}_0 x(\tau) y(\tau)$, and normalizing the result by the final size, $\int i(t) dt$. To compute the between cohort variance, we integrated the squared cohort mean $\mu^2(\tau)$ weighted by the incidence $i(\tau)$ and divided it by the final size. The result minus the squared mean of $\mathcal{R}_c$ equaled the between cohort variance. The within-cohort variance was computed by taking integral over the cohorts' variance $\sigma^2(\tau)$ weighted by the incidence. The result was also normalized by the final size. The total variance was the sum of the between- and within-variances. We also calculated the total variance independently; we integrated $\mathbb{E}[\mathcal{R}_c^2(\tau)]$ weighted by the incidence and divided it by the final size. The result minus the squared mean of $\mathcal{R}_c$ yielded the total variance. Both approaches produced the same value for the total variance.

We used deterministic simulations to compute the y-axis values in the panels b and c of Figure S1. Specifically, at each time point t_f, we considered the

cohorts infected up to time t_f, including the initial infectious cohort. We then computed cohort statistics by calculating the cases generated by these cohorts up to time t_f. For example, for a cohort infected at time point $\tau < t_f$, the mean, $\mu(\tau, t_f)$, and variance of $\mathcal{R}_c$, $\sigma^2(\tau, t_f)$, read as:

$$\mu(\tau, t_f) = \int_{t=\tau}^{t_f} f(t-\tau)dt \int_{\tau}^{t} \mathcal{R}_0 x(s)ds + F(t_f - \tau) \int_{\tau}^{t_f} \mathcal{R}_0 x(s)ds, \quad (10a)$$

$$\mathbb{E}[\mathcal{R}_c^2(\tau, t_f)] = \int_{t=\tau}^{t_f} f(t-\tau)dt \left(\int_{\tau}^{t} \mathcal{R}_0 x(s)ds \right)^2 + F(t_f - \tau) \left(\int_{\tau}^{t_f} \mathcal{R}_0 x(s)ds \right)^2, \quad (10b)$$

$$\sigma^2(\tau, t_f) = \mathbb{E}[\mathcal{R}_c^2(\tau, t_f)] - \mu^2(\tau, t_f), \quad (10c)$$

where $F(\cdot)$ is the survival function associated with the infectious period density $f(\cdot)$. The mean and variance of $\mathcal{R}_c$ are then

$$\mu(t_f) = \frac{\int_0^{t_f} i(\tau)\mu(\tau, t_f)d\tau}{\int_0^{t_f} i(t)dt}, \quad (11a)$$

$$\sigma^2(t_f) = \frac{\int_0^{t_f} i(\tau)\mathbb{E}[\mathcal{R}_c^2(\tau, t_f)]d\tau}{\int_0^{t_f} i(t)dt} - \mu^2(t_f). \quad (11b)$$

The between-cohort component of κ at time point t_f is $\kappa_{\text{bet}}(t_f) = \sigma^2_{\text{bet}}(t_f)/\mu^2(t_f)$, where

$$\sigma^2_{\text{bet}}(t_f) = \frac{\int_0^{t_f} i(\tau)\mu(\tau, t_f)^2}{\int_0^{t_f} i(t)dt} - \mu^2(t_f).$$

To generate panels (b) and (c) of Figure S2, as with the corresponding panels of Figure S1, at each time point t_f, we considered only cohorts infected by t_f, including the initial infectious cohort. Instead, we counted the total cases caused by these cohorts during the outbreak. To do so, we simply substituted $\mu(\tau)$ and $\mathbb{E}[\mathcal{R}_c^2(\tau)]$ for $\mu(\tau, t_f)$ and $\mathbb{E}[\mathcal{R}_c^2(\tau, t_f)]$ in Equation (11), where $\mu(\tau)$ and $\mathbb{E}[\mathcal{R}_c^2(\tau)]$ were provided in Equation (9).

All integrations were done using `lsoda` method with the `R` package `deSolve`. All simulations were carried out using `R 4.5.2`. Code for all numerical simulations is housed at: `https://github.com/dushoff/kappaCode`. We solved all integrals across a range of values for $\mathcal{R}_0$, using the starting values $y_0 = 10^{-9}; x_0 = 1 - 10^{-9}$ to represent the limiting case in which there are no exogenous cases. In building these simulations, we used a range of time step sizes, noting convergence towards known and conjectured values (e.g., epidemic final size, mean case reproduction number, variance in case reproduction number) as resolution increased.

S.1 Analytical Results

The standard SIR model is described by the following equations:

$$\dot{x}(t) = -\mathcal{R}_0 x(t) y(t),$$
$$\dot{y}(t) = \mathcal{R}_0 x(t) y(t) - y(t),$$

where $x(t)$ and $y(t)$ denote the proportion of susceptible and infectious individuals, respectively. Time has been normalized by the recovery rate. Let $i(t)$ denote the incidence $\mathcal{R}_0 x(t) y(t)$, and define the size of the epidemic as follows:

$$Z = \int_0^\infty dt i(t).$$

Let $f(\cdot)$ denote the probability density function of the infectious period. Define $F(t)$ as the corresponding survival distribution, that is

$$F(t) = 1 - \int_0^t d\tau f(\tau) = \int_t^\infty d\tau f(\tau). \tag{S.1}$$

Note that $F(t)$ is the probability that an individual infected at time 0 remains infectious after time t.

We define the *expected case reproductive number* between time τ and ρ as

$$C(\tau, \rho) = \mathcal{R}_0 \int_\tau^\rho dt\ x(t).$$

The kth weighted raw moment of C is then defined as

$$C_k = \int_0^\infty d\tau \int_\tau^\infty d\rho\ w(\tau, \rho)(C(\tau, \rho))^k, \tag{S.2}$$

where $w(\tau, \rho) = i(\tau) f(\rho - \tau)$ is the size of the cohort that became infectious at time τ and recovered at time ρ.

In Section S.1.1, we prove the following statement: The excess variation of the individual case reproductive number in the standard SIR model outbreak is 1, provided that the initially infectious cohort size is negligible. Mathematically, this amounts to proving that

$$\kappa = \frac{C_0 C_2}{C_1^2} - 1 = 1. \tag{S.3}$$

S.1.1 Proof

We will calculate C_0, C_1 and C_2. We can calculate C_0 and C_1 without making any assumptions about the functional form of f (we don't have to assume that the wait times are exponentially distributed). We do need F to be memoryless, i.e., obey $F(a + b) = F(a)F(b)$, to get the "right" value of C_2. Calculating each

of these moments essentially involves multiple manipulations of the limits of the integrals involved. To proceed, we assume that size of the initial infectious cohort is negligible.

From Equation S.2, we have

$$\begin{aligned} C_0 &= \int_0^\infty d\tau \int_\tau^\infty d\rho \; w(\tau,\rho) \\ &= \int_0^\infty d\tau \int_\tau^\infty d\rho \; i(\tau) f(\rho-\tau) \end{aligned}$$

Substituting $\omega + \tau$ for ρ in the inner integral, results in

$$C_0 = \int_0^\infty d\tau \; i(\tau) \int_0^\infty d\omega \; f(\omega).$$

Since f is the residence time distribution, the inner integral is simply 1. It follows that

$$\begin{aligned} C_0 &= \int_0^\infty d\tau \; i(\tau) \\ &= Z. \end{aligned}$$

The next step is to prove that $C_1 = Z$. From Equation S.2, we have

$$\begin{aligned} C_1 &= \int_0^\infty d\tau \int_\tau^\infty d\rho \; w(\tau,\rho) C(\tau,\rho) \\ &= \mathcal{R}_0 \int_0^\infty d\tau \int_\tau^\infty d\rho i(\tau) f(\rho-\tau) \int_\tau^\rho dt x(t). \end{aligned}$$

Instead of first integrating over t from τ to ρ and then over ρ from τ to ∞, we can first integrate over ρ from t to ∞ and then integrate over t from τ to ∞, i.e.,

$$C_1 = \mathcal{R}_0 \int_0^\infty d\tau \int_\tau^\infty dt i(\tau) x(t) \int_t^\infty d\rho f(\rho-\tau).$$

Substitute $\rho = \omega + \tau$ in the innermost integral and using Equation S.1, we get

$$\begin{aligned} C_1 &= \mathcal{R}_0 \int_0^\infty d\tau \int_\tau^\infty dt \; i(\tau) x(t) \int_{t-\tau}^\infty d\omega f(\omega) \\ &= \mathcal{R}_0 \int_0^\infty d\tau \int_\tau^\infty dt \; i(\tau) x(t) F(t-\tau). \end{aligned}$$

Note again that we have not used the fact that f is exponential, we have merely used the definition of F as an integral of f. By interchanging the order of the integrals once more, we have

$$\begin{aligned} C_1 &= \mathcal{R}_0 \int_0^\infty d\tau \int_\tau^\infty dt \; i(\tau) x(t) \int_{t-\tau}^\infty d\omega f(\omega) \\ &= \mathcal{R}_0 \int_0^\infty dt \; x(t) \int_0^t d\tau \; i(\tau) F(t-\tau). \end{aligned}$$

Note that the inner integrand counts the number of individuals that entered the infectious state at some time $\tau < t$ and remain infectious at time t. When we integrate over all times $\tau < t$, we simply get the number of individuals who are infections at time t, i.e., $y(t)$. So,

$$C_1 = \mathcal{R}_0 \int_0^\infty dt\ x(t) \int_0^t d\tau\ i(\tau) F(t-\tau) \tag{S.4}$$

$$= \mathcal{R}_0 \int_0^\infty dt\ x(t) y(t) \tag{S.5}$$

$$= \int_0^\infty dt\ i(t) \tag{S.6}$$

$$= Z. \tag{S.7}$$

The next step is to show $C_2 = 2Z$. Given Equation S.2, we have

$$C_2 = \int_0^\infty d\tau \int_\tau^\infty d\rho\ w(\tau,\rho) \left(C(\tau,\rho)\right)^2$$

$$= \mathcal{R}_0^2 \int_0^\infty d\tau \int_\tau^\infty d\rho\ i(\tau) f(\rho-\tau) \left(\int_\tau^\rho dt x(t) \right)^2$$

Now we break down the square integral into a double integral

$$\left(\int_\tau^\rho x(t) dt \right)^2 = \left(\int_\tau^\rho x(t) dt \right) \left(\int_\tau^\rho x(s) ds \right)$$

$$= \left(\int_\tau^\rho x(t) dt \right) \left(\int_\tau^\rho x(s) ds \right)$$

$$= \int_\tau^\rho \int_\tau^\rho dt ds\ x(t) x(s).$$

In $t - s$ space, this is integrating $x(t)x(s)$ over a square region whose vertices are (τ,τ), (τ,ρ), (ρ,τ) and (ρ,ρ). This is twice the integral over the triangle whose vertices are (τ,τ), (τ,ρ) and (ρ,τ). So,

$$\left(\int_\tau^\rho x(t) dt \right)^2 = 2 \int_\tau^\rho ds \int_\tau^s dt\ x(t) x(s)$$

Plugging this back into the expression for C_2 yields

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty d\tau \int_\tau^\infty d\rho \int_\tau^\rho ds \int_\tau^s dt\ i(\tau) f(\rho-\tau) x(t) x(s).$$

First, we reorder the integrals over ρ and s and change the limits appropriately

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty d\tau\ i(\tau) \int_\tau^\infty ds\ x(s) \int_s^\infty d\rho \int_\tau^s dt f(\rho-\tau) x(t).$$

Next, we reorder the integrals over ρ and t to get

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty d\tau\ i(\tau) \int_\tau^\infty ds\ x(s) \int_\tau^s dt\ x(t) \int_s^\infty d\rho f(\rho - \tau).$$

By substituting $\rho = \omega + \tau$ and using Equation S.1, we get

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty d\tau\ i(\tau) \int_\tau^\infty ds\ x(s) \int_\tau^s dt\ x(t) F(s - \tau).$$

We then reorder the integrals over τ and s

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty ds\ x(s) \int_0^s d\tau \int_\tau^s dt\ x(t) i(\tau) F(s - \tau).$$

Swapping the integrals over τ and t, results in

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty ds\ x(s) \int_0^s dt\ x(t) \int_0^t d\tau\ i(\tau) F(s - \tau).$$

Now, we use the fact that the survival function F is memoryless, i.e., $F(a+b) = F(a)F(b)$, and replace $F(s-\tau)$ with $F(s-t)F(t-\tau)$

$$C_2 = 2\mathcal{R}_0^2 \int_0^\infty ds\ x(s) \int_0^s dt\ x(t) \int_0^t d\tau\ i(\tau) F(s - t) F(t - \tau).$$

Next, we use the same argument we used to go from Equation S.4 to Equation S.5, which results in

$$\begin{aligned}
C_2 &= 2\mathcal{R}_0^2 \int_0^\infty ds\ x(s) \int_0^s dt\ x(t) F(s - t) y(t) \\
&= 2\mathcal{R}_0 \int_0^\infty ds\ x(s) \int_0^s dt\ i(t) F(s - t) \\
&= 2\mathcal{R}_0 \int_0^\infty ds\ x(s) y(s) \\
&= 2 \int_0^\infty ds\ i(s) \\
&= 2Z.
\end{aligned}$$

Finally, replacing C_0, C_1, and C_2 by Z, Z, and $2Z$, respectively, in Equation S.3, results $\kappa = 1$.

S.1.2 Incorporating the Size of Initial Infections

In the previous section, Equation S.3 was proved under the assumption that the initially infectious population was negligibly small. In what follows, we revisit the results while relaxing this assumption. To account for the initial infections, we assume that the epidemic starts abruptly at time $t = 0$, i.e.,

$$i(t) = y_0 \delta(t), \quad t \in (-\infty, 0]. \tag{S.8}$$

Here, y_0 denotes the proportion of the population that were infectious at the beginning of the outbreak, $t = 0$, and $\delta(t)$ denotes the Dirac delta distribution [2]. Then, accounting for the initially infectious cohort, the proportion of the population that is infectious at t is given by

$$y(t) = \int_{0^-}^{t} d\tau \; i(\tau) F(t-\tau).$$

Here, the lower limit of integration is 0^- to include the contribution of the Dirac delta distribution at $t = 0$. The total weight C_0 will now includes the initially infectious cohort size as well, i.e,

$$C_0 = \int_{0^-}^{\infty} dt \; i(t) \tag{S.9}$$

The first raw moment C_1 is given by

$$\begin{aligned}
C_1 &= \int_{0^-}^{\infty} d\tau \int_{\tau}^{\infty} d\rho \; w(\tau,\rho) C(\tau,\rho) \\
&= \mathcal{R}_0 \int_{0^-}^{\infty} d\tau \int_{\tau}^{\infty} dt i(\tau) x(t) \int_{t}^{\infty} d\rho f(\rho-\tau) \\
&= \mathcal{R}_0 \int_{0^-}^{\infty} d\tau \int_{\tau}^{\infty} dt i(\tau) x(t) F(t-\tau) \\
&= \mathcal{R}_0 \int_{0}^{\infty} dt x(t) \int_{0^-}^{t} d\tau i(\tau) F(t-\tau) \\
&= \mathcal{R}_0 \int_{0^-}^{\infty} dt \; x(t) y(t).
\end{aligned}$$

Given both $x(t)$ and $y(t)$ are bounded, without loss of generality, the lower limit can be replaced with 0^+. Thus, the first raw moment C_1 becomes

$$\begin{aligned}
C_1 &= \mathcal{R}_0 \int_{0^+}^{\infty} dt \; x(t) y(t) \\
&= \int_{0^+}^{\infty} dt \; i(t).
\end{aligned}$$

We note that C_1 does not include the size of initially infectious cohort. Now we move on to recalculate C_2, while taking the initially infectious cohort into account. We have

$$\begin{aligned}
C_2 &= \int_{0^-}^{\infty} d\tau \int_{\tau}^{\infty} d\rho \; w(\tau,\rho) \left(C(\tau,\rho)\right)^2 \\
&= \mathcal{R}_0^2 \int_{0^-}^{\infty} d\tau \int_{\tau}^{\infty} d\rho \; i(\tau) f(\rho-\tau) \left(\int_{\tau}^{\rho} dt x(t) \right)^2
\end{aligned}$$

Using the same mathematical manipulation as in section S.1.1, we obtain

$$\begin{aligned} C_2 &= 2\mathcal{R}_0^2 \int_{0^-}^{\infty} ds\, x(s) \int_{0^-}^{s} dt\, x(t) \int_{0^-}^{t} d\tau\, i(\tau) F(t-\tau) F(s-t) \\ &= 2\mathcal{R}_0^2 \int_{0^-}^{\infty} ds\, x(s) \int_{0^-}^{s} dt\, x(t) y(t) F(s-t) \end{aligned}$$

We split the innermost integral into two integrals

$$\begin{aligned} C_2 &= 2\mathcal{R}_0^2 \int_{0^-}^{\infty} ds\, x(s) \int_{0^-}^{s} dt\, x(t) y(t) F(s-t) \\ &= 2\mathcal{R}_0^2 \int_{0^-}^{\infty} ds\, x(s) \left(\underbrace{\int_{0^-}^{0^+} dt\, x(t) y(t) F(s-t)}_{0} + \int_{0^+}^{s} dt\, x(t) y(t) F(s-t) \right) \\ &= 2\mathcal{R}_0^2 \int_{0^-}^{\infty} ds\, x(s) \int_{0^+}^{s} dt\, x(t) y(t) F(s-t) \\ &= 2\mathcal{R}_0 \int_{0^-}^{\infty} ds\, x(s) \int_{0^+}^{s} dt\, i(t) F(s-t). \end{aligned}$$

The innermost integral gives the proportion of the population who entered the infectious state at some time $t > 0$ and remain infectious at time s. Hence, it equals the total proportion infectious at time s minus the proportion of the population who were initially infectious and remain infectious at time s, i.e., $y(s) - y_0 F(s)$. Having replaced the innermost integral with $y(s) - y_0 F(s)$, we obtain

$$\begin{aligned} C_2 &= 2\mathcal{R}_0 \int_{0^-}^{\infty} ds\, x(s) \left(y(s) - y_0 F(s) \right) \\ &= 2\mathcal{R}_0 \int_{0^-}^{\infty} ds\, x(s) y(s) - 2\mathcal{R}_0 y_0 \int_{0^-}^{\infty} ds\, x(s) F(s) \\ &= 2 \int_{0^+}^{\infty} i(s) ds - 2\mathcal{R}_0 y_0 \int_{0^+}^{\infty} ds\, x(s) F(s) \\ &= 2C_1 - 2\mathcal{R}_0 y_0 \int_{0^+}^{\infty} ds\, x(s) F(s). \end{aligned}$$

Note that changing the lower limit of integration from 0^- to 0^+ does not affect the result because the integrand is bounded near $t = 0$. The second term in the right hand side is the twice the expected infectiousness of the initial cohort. Here we notice that the second raw moment is *dependent* on the key parameter $\mathcal{R}_0$. However, as the initial cohort size y_0 tends to zero, (i) the difference between C_2 and twice the first raw moment $2C_1$ approaches zero; (ii) the total weight C_0 (S.9) converges to C_1. Thus, Equation S.3 is retrieved.

S.1.3 Moments of time-truncated case reproductive number at some point during the outbreak

In this section, we derive the moments of the time-truncated case reproductive number at a given time $j \in (0, \infty)$ during the outbreak, accounting for the initially infectious cohort. Up to time j, the expected realized infections due to individuals who became infectious at time $\tau \in (0, j)$ and remain infectious at time j is the equal to that of the individuals who became infectious at τ and recovered at time j. To capture this observation, we introduce a modified infectious period density function, denoted $g_j(t, \tau)$. This function equals the true infectious period density function $f(t-\tau)$ for $t < j$, and assigns the remaining probability mass to $t = j$. Hence, its cumulative distribution function reaches 1 at j, i.e.,

$$g_j(t,\tau) = \begin{cases} f(t-\tau), & \text{if } t < j \text{ and } \tau < j, \\ F(j-\tau)\delta(t-j), & \text{if } t \geq j \text{ and } \tau < j, \\ 0, & \text{otherwise.} \end{cases}$$

Accordingly, we redefine the size of the cohort that became infectious at time τ and recovered at time ρ as follows

$$w_j(\tau, \rho) = i(\tau) g_j(\rho, \tau), \quad \tau \leq \rho \leq j.$$

Note that $\int_t^j g_j(s, \tau)\, ds$ is equal to $F(t-\tau)$ for $t < j$. It is straightforward to show that the total weight $C_0(j)$ equals

$$C_0(j) = \int_{0^-}^{j} dt\; i(t). \tag{S.10}$$

The first raw moment $C_1(j)$ is given by

$$\begin{aligned} C_1(j) &= \int_{0^-}^{j} d\tau \int_{\tau}^{j} d\rho\; w_j(\tau,\rho) C(\tau,\rho) \\ &= \mathcal{R}_0 \int_{0^-}^{j} d\tau \int_{\tau}^{j} d\rho\; i(\tau) g_j(\rho,\tau) \int_{\tau}^{\rho} dt x(t) \\ &= \mathcal{R}_0 \int_{0^-}^{j} d\tau \int_{\tau}^{j} dt\; i(\tau) x(t) \int_{t}^{j} d\rho g_j(\rho,\tau) \\ &= \mathcal{R}_0 \int_{0^-}^{j} d\tau \int_{\tau}^{j} dt\; i(\tau) x(t) F(t-\tau) \\ &= \mathcal{R}_0 \int_{0}^{j} dt x(t) \int_{0^-}^{t} d\tau i(\tau) F(t-\tau) \\ &= \mathcal{R}_0 \int_{0^-}^{j} dt\; x(t) y(t) \\ &= \int_{0^+}^{j} dt\; i(t). \end{aligned}$$

Now we move on to calculate $C_2(j)$. We have

$$\begin{aligned}
C_2(j) &= \int_{0^-}^{j} d\tau \int_{\tau}^{j} d\rho \; w_j(\tau,\rho) \left(C(\tau,\rho)\right)^2 \\
&= \mathcal{R}_0^2 \int_{0^-}^{j} d\tau \int_{\tau}^{j} d\rho \; i(\tau) g_j(\rho,\tau) \left(\int_{\tau}^{\rho} dt x(t) \right)^2 \\
&= 2\mathcal{R}_0^2 \int_{0^-}^{j} ds \; x(s) \int_{0^-}^{s} dt \; x(t) \int_{0^-}^{t} d\tau \; i(\tau) F(s-t) F(t-\tau) \\
&= 2\mathcal{R}_0^2 \int_{0^-}^{j} ds \; x(s) \int_{0^-}^{s} dt \; x(t) y(t) F(s-t) \\
&= 2\mathcal{R}_0^2 \int_{0^-}^{j} ds \; x(s) \int_{0^-}^{s} dt \; x(t) y(t) F(s-t)
\end{aligned}$$

Following the same steps as in Section S.1.2, we obtain

$$\begin{aligned}
C_2(j) &= 2\mathcal{R}_0 \int_{0^-}^{j} ds \; x(s) \int_{0^+}^{s} dt \; i(t) F(s-t) \\
&= 2\mathcal{R}_0 \int_{0^-}^{j} ds \; x(s) \left(y(s) - y_0 F(s) \right) \\
&= 2\mathcal{R}_0 \int_{0^-}^{j} ds \; x(s) y(s) - 2\mathcal{R}_0 y_0 \int_{0^-}^{j} ds \; x(s) F(s) \\
&= 2 \int_{0^+}^{j} i(s) \; ds - 2\mathcal{R}_0 y_0 \int_{0}^{j} ds \; x(s) F(s) \\
&= 2C_1(j) - 2\mathcal{R}_0 y_0 \int_{0}^{j} ds \; x(s) F(s).
\end{aligned}$$

The second raw moment is *dependent* on the key parameter $\mathcal{R}_0$. However, as the initial cohort size y_0 tends to zero, both $C_2(j) - 2C_1(j)$ and $C_0(j) - C_1(j)$ approach zero. Hence, $\kappa(j)$, defined as

$$\kappa(j) = \frac{C_0(j) C_2(j)}{C_1^2(j)} - 1,$$

approaches 1.

Supplementary Figures

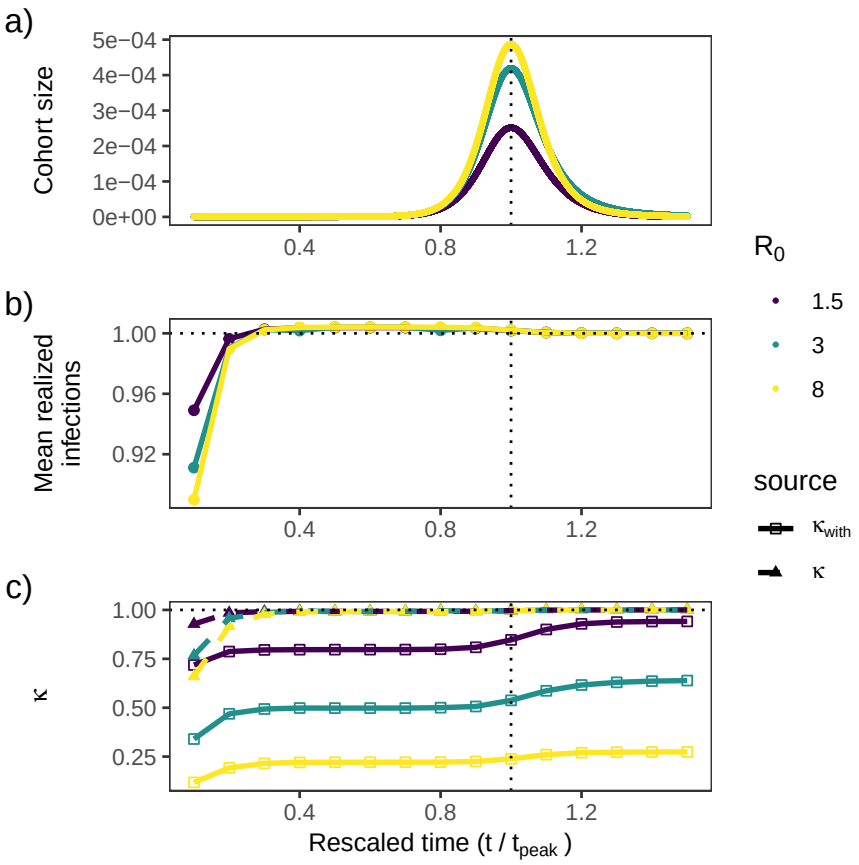


Figure S1: **Mean and heterogeneity of realized infections evolve over the course of an outbreak.** Panel a: The size of the cohort infected during $[t, t + \Delta t]$ is given by the area under the incidence curve over that interval, where $\Delta t = 10^{-4} t_{\text{peak}}$ and t_{peak} denotes the time of peak incidence. Panel b: At each time point, mean realized infections is computed using the cases had occurred by that time. Early in the outbreak, the initial infectious cohort constitutes a relatively large fraction of all infected population up to that time. Because we assume a "jump-start" outbreak, the infectors of this initial cohort are not included, and hence, the mean realized infections is noticeably below one. At early times ($t \lesssim 0.4 t_{\text{peak}}$), epidemics with larger $\mathcal{R}_0$ have accumulated fewer infections, making the effect of this omission more pronounced. Panel c: κ of realized infections increases over the course of the outbreak. For epidemics with smaller $\mathcal{R}_0$, a given fraction of t_{peak} corresponds to a longer elapsed time, allowing differences in infectious periods among cohort members to become apparent. As a result, the within component of κ (measured as the ratio of within-cohort variance to the squared mean) is larger than that for epidemics with larger $\mathcal{R}_0$. For panels b and c, the simulation was truncated at each plotted time. The y-axis value at each time point was computed by assigning to each cohort their realized number of infections up to that time. In all panels, time units measured in the mean infectious period are rescaled relative to the peak time, so that peak incidence occurs at $(t/t_{\text{peak}} = 1)$. The outbreaks were modeled using standardized compartmental models.

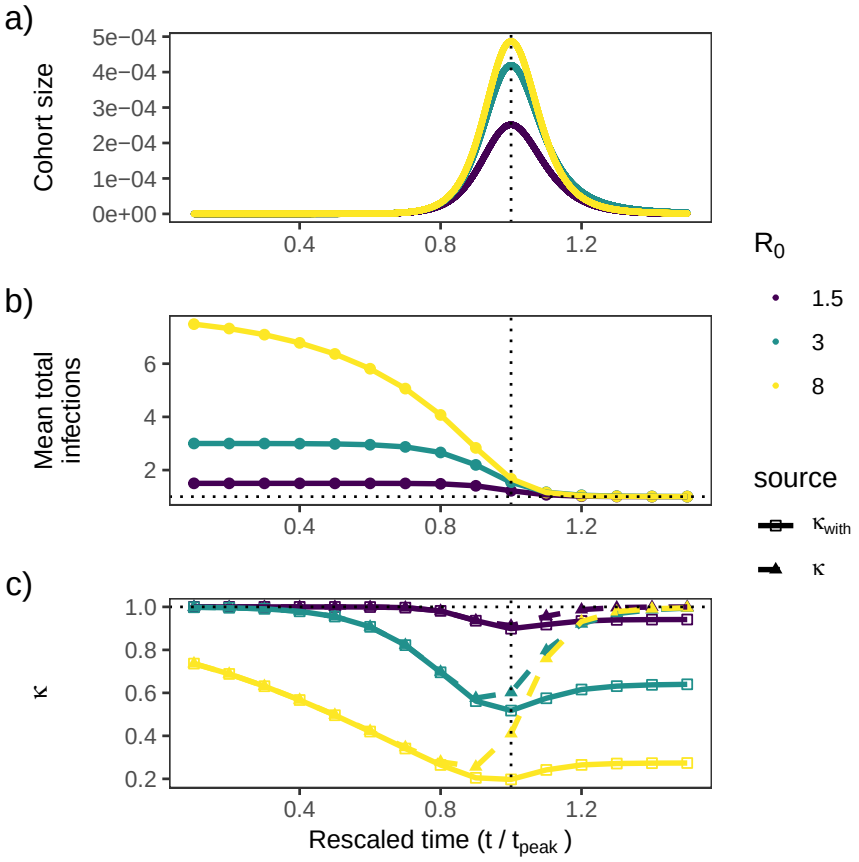


Figure S2: **Forecasts of the distribution of total infections evolve as an outbreak unfolds.** Panel a: Cohort size infected during $[t, t + \Delta t]$ is measured as the area under the incidence curve over this interval ($\Delta t = 10^{-4} t_{\text{peak}}$, where t_{peak} is the time of peak incidence). Panel b: At each time point, mean total infections is calculated across all cohorts infected by that time, including the initial infectious cohort of size 10^{-9}, while accounting for each cohort's subsequent contributions to disease transmission. Cohorts infected earlier have higher mean total infections, with values depending on key parameter $\mathcal{R}_0$. As incidence increases, later-infected cohorts, who have larger sizes, encounter susceptible pool depletion, decreasing mean total infections. After the peak, the susceptible population size stabilizes, and the mean approaches one. Panel c: The within-cohort component of the squared coefficient of variation κ_{with}, measured as the ratio of the within-cohort variance to the squared mean, is the main driver of heterogeneity among the early-infected cohorts, as they experience a similar size of susceptible population. As the outbreak unfolds, the decline in the susceptible population drives between-cohort variation, which is more pronounced in epidemics with large $\mathcal{R}_0$. κ reaches its minimum around the outbreak peak; large cohorts experience sharp depletion of susceptibles, and the impact of variation in recovery time fades, declining the variance, and hence κ. After the peak, κ rebounds to one. For panels b and c, the y-axis value at each plotted time point was quantified using only the cohorts that had been infected up to that time. The outbreaks were modeled using standardized compartmental models.

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